

How to Use This Document

This handout highlights the new 2025 recommendations from ILCOR (AHA and ERC) that apply to the material you will learn in your current 8th Edition First Aid, CPR & AED course. It is organized by module so you can follow along easily during your class and understand what has been updated.

These updates reflect the most current international guidelines and will be part of the upcoming 9th Edition. If you have questions during the course, please ask your instructor.

2025 ILCOR - First Aid/CPR/AED Treatment Recommendations Updates – Participant Learning Guide			
Module	Topic / Subtopic	2025 ILCOR Change / Update	PW Location
General	Inclusive & accessible training	Training materials and first aid assistance should be accessible, available in multiple languages, and supported with reasonable accommodations when needed (ADA/508).	NEW
	Terminology & Inclusive Language	Use the term “person” instead of “victim,” “PPE (personal clear, inclusive language.	
Course Introduction	Definition of First Aid	First Aid is the immediate assistance in case of a medical emergency until professional help is available.	1
	Guiding Factors	The four guiding factors in a first aid situation are: 1. The person, 2. The provider, 3. The treatment and 4. The environment.	
	Goals	The three standard goals of first aid are: 1. Preserve Life 2. Prevent Deterioration 3. Promote Recovery	
Module 1: Acting in an Emergency	Risk-based first-aid planning	Workplaces must align their first aid kits, training, and drills with the specific hazards of the job. AEDs must be easy to see and checked monthly to ensure they are ready for use.	1 - 4
	Team Roles	Lay responder roles must be clearly assigned: scene leader, EMS/911 caller, AED retriever, CPR provider, bleeding controller, and EMS guide.	
	Preparedness & policies	Workplaces should conduct safety huddles and brief emergency drills, ensure AEDs are visible and maintained monthly, and provide naloxone in settings where overdose risk exists.	
	First Aid Kit	First aid kits must be risk-based, checked regularly, and aligned with the ANSI/ISEA Z308.1 standard to ensure workplaces have the right supplies.	
	Priorities & Activation	The priority sequence of giving first aid is: 1. Ensure personal safety first 2. Activate EMS/911 3. Provide care	
Module 2: Preventing Disease Transmission	Infection Control	There is a stronger focus on full infection control. Participants should know how to: • Put on and remove PPE correctly, • Follow good respiratory hygiene, • Use alcohol-based hand rub safely, and • Clean up biohazard materials properly to prevent disease transmission.	5, 6

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Module 3: Check the Person	Primary Assessment Order	The new primary assessment sequence is: 1. Check responsiveness 2. Assess breathing within 10 seconds 3. Identify and control severe bleeding immediately	7 - 10
	Tap & Shout	The recommended way to check responsiveness is now the “tap and shout” technique—tap the person’s shoulders and shout to see if they respond.	
	Dispatcher-Assisted Recognition:	Dispatchers can now help identify cardiac arrest over the phone. Lay rescuers should listen carefully to dispatcher instructions, especially when the person has agonal breathing (gasping or irregular breaths), which is a sign of cardiac arrest.	
Module 4: Recovery Position	Indications & Monitoring	Use the recovery position only if the person is breathing normally and no spinal injury is suspected. Always monitor their breathing. Return the person to a supine position if breathing worsens or becomes unclear. If the person is found face-down or bent over, first place them supine to prevent positional asphyxia.	11, 12
Module 5: Bleeding and Wound Care	Control Sequence for Bleeding	The new ABC sequence for bleeding control is: A – Alert (call 911 and get the bleeding kit), B – Bleeding Source (find the source quickly), and C – Control (apply pressure, pack the wound, or use a tourniquet). This sequence emphasizes calling EMS early.	13, 14
	Soaked dressings	Do not remove soaked dressings; instead, add more layers on top and keep firm pressure.	15, 16
	Pack the wound	Use “pack the wound” for severe bleeding; pack deep into the wound instead of using a simple bandage.	15, 16
	Tourniquet time & shock monitoring	Record the exact time the tourniquet was applied and monitor for shock; keep firm pressure if a tourniquet can’t be used.	22, 23
	Hemostatic dressings	Use hemostatic gauze/dressings for deep or severe bleeding that does not stop with pressure.	15, 16
	Limb elevation / Pressure points	Do not use limb elevation or pressure points—these techniques are no longer recommended.	

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Module 5: Bleeding and Wound Care	Nosebleeds (epistaxis)	Pinch the soft part of the nose, lean forward, apply pressure for 10 minutes, and avoid tilting the head back.	19, 26, 27
	Eye Injuries	Protect the injured eye without pressure, cover loosely, and seek urgent evaluation if vision changes.	
	Mouth/teeth injuries	For a knocked-out tooth, place it in milk or saline and get immediate dental care.	19, 20
	Penetrating and Puncturing Injuries	Do not remove embedded objects; stabilize them with bulky dressings and apply pressure around the wound; call EMS.	18
	Amputations	Stop bleeding first, then wrap the amputated part in moist gauze, place in a bag, cool (not freeze), label it, and activate EMS.	25
	Internal bleeding & shock	Recognize key signs (abdominal pain, bruising, blood vomit/cough, restlessness, cool-pale skin); call EMS early and keep the person comfortable with slight leg elevation only if no trauma.	28 - 30
Module 7: Burns	Cool–Cover–Call	Use the simplified sequence Cool → Cover → Call to start all burn care.	31 - 35
	Stop the burning source	Stop the burning source first before giving any care.	
	Burn Classifications by Depth	Use the updated depth terms: superficial, partial-thickness, full-thickness; appearance (not degree numbers) shows severity.	
	Burn Types by Cause	Identify cause: thermal, chemical, or electrical, since it guides safety and care.	32 - 35
	Thermal Burns - Updated Care Steps	"Cool the burn, not the person"; use clean running water for 10–20 min, remove jewelry/tight clothing, and avoid ice, creams, or oils.	33
	Chemical Burns	Ensure safety and PPE; brush off dry chemicals; flush with water ≥20 minutes; remove contaminated clothing; no neutralizers; call EMS for large areas or if eyes/inhalation involved.	33
	Electrical Burns	Turn power off first; look for entry/exit wounds; do not cool or apply ointments; call EMS, check breathing, and start CPR/AED if needed.	

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Module 8: Bone, Joint and Muscle Injuries	Plain terminology	Use “broken bone” instead of “fracture” in basic first aid.	36
	Unified musculoskeletal care	Sprains, strains, and broken bones all receive the same initial first aid.	
	Recognizing Injuries	Look for the cause of injury and visible signs; limit movement; all dislocations should be immobilized; severe forces may mean internal injury or shock.	40 - 43
	Splinting and Immobilization	Follow: assess → stabilize → splint if trained → cold pack 20 min; keep the injured part as found, and check circulation before/after.	38 - 40
	Soft-tissue injuries & cramps	Use gentle stretching, hydrate, apply warmth after a spasm or a cold pack if swelling appears, and return to activity gradually.	42, 43
Module 9: Head and Spine Injuries	Spinal care approach	Avoid routine immobilization; keep minimal movement and use manual support. Move the person only if the airway is compromised or there is danger.	45, 46
	Spinal care -When to Call 911	Call 911 if the person is unresponsive, not breathing normally, has a suspected spine injury (high fall, crash, sports impact), or has numbness, tingling, weakness, or inability to move limbs.	49
	Head injury red flags	Watch for worsening headache, unequal pupils, repeated vomiting, or confusion/altered mental status.	44 - 47
	Helmet removal	Remove the helmet only if needed for airway access or CPR; otherwise, keep it on.	
	Concussion monitoring	Monitor symptoms; avoid strenuous activity or work until a medical professional authorizes return.	
Module 10: Sudden Illnesses	Terminology and Rapid Recognition of Emergency	Use the updated term “medical emergency caused by illness” and focus on recognizing symptoms early and activating EMS quickly.	50, 51
	General Signs & Symptoms of Medical Emergencies	Look for easy-to-spot warning signs: confusion, slurred speech, weakness, sweating. Recognize and call EMS – do not diagnose or give medications.	
	Heart attack	You may give aspirin only if the person is not allergic and has no bleeding risk. Call EMS first.	51, 52, 57
	Stroke	Use BE FAST (Balance, Eyes, Face, Arm, Speech, Time) for recognition. Note the time symptoms began and report to EMS.	52, 53
	Asthma & breathing difficulty	Use BE FAST (Balance, Eyes, Face, Arm, Speech, Time) for recognition. Note the time symptoms began and report to EMS.	53, 54

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Module 10: Sudden Illnesses	Diabetic Emergencies	Give 15–20 g of fast-acting sugar (juice, glucose gel, tablets). Do not use candy bars or soda as primary treatment.	54
	Fainting (syncope)	If unresponsive for >1 minute or not breathing normally, start CPR. Lay flat; elevate legs if safe; monitor.	55
	Abdominal pain – red flags	No food, drink, or medication by mouth. Seek early medical care if pain is severe or getting worse.	56
	Seizures	If the person is not breathing normally after a seizure, start CPR.	57
	Mental health first aid	Recognize signs of acute emotional distress or possible suicidal risk. Keep the person safe, stay with them if possible, and connect them to professional help immediately (EMS, crisis line, or workplace mental health resources).	NEW
Module 11: Poisoning and Allergic Reactions	Poison routes	Poisoning can happen by ingestion, inhalation, absorption, or injection.	59
	Anaphylaxis – epinephrine	Use epinephrine immediately; give a second dose after 5 minutes if symptoms continue. Call EMS right away – do not wait for multiple symptoms.	61, 62
	Opioid emergencies – naloxone	Follow the sequence: Call 911 → Start CPR (30:2) → Apply AED → Give naloxone → Continue CPR.	63, 64
	Absorbed poisons (skin/eye)	Flush with water for at least 15 minutes. Wear PPE and do not use neutralizers. Call Poison Control or EMS if symptoms continue.	60
	Poison ivy/oak/sumac	Wash with soap and water; avoid scratching. Antihistamines may help if allowed at the workplace.	65
	Injected poisons (bites/stings/animal or human)	Do not cut or suck the area. Scrape out the stinger gently. Watch for signs of severe allergic reaction. Keep the affected limb lower than the heart.	60
	Ingested Poisons	Do not give food or water unless Poison Control instructs you. At work, consider whether cleaning products or chemicals may be involved.	60
	Inhaled poisons / CO	Ensure rescuer safety. Move the person to fresh air. Think of CO poisoning if there is headache, dizziness, or confusion.	60

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Module 12: Cold and Heat Emergencies	Heat exhaustion vs heat stroke	Clear differentiation; rapid cooling for heat stroke. When in doubt, treat as heat stroke.	68 - 72
	Hypothermia & frostbite	Remove wet clothing; gradual rewarming; never rub frozen tissue.	
	Hypothermia Level	Clarifies mild vs. severe hypothermia based on shivering and mental status.	
	Hypothermia – CPR considerations	If the person is not breathing normally, start CPR/AED early – even cold victims can survive.	
Module 14: CPR and AED	Chain of Survival	Unified Chain of Survival now has six links: 1. Recognize the emergency & activate EMS 2. High-quality CPR 3. Early AED 4. Advanced care 5. Post-cardiac arrest care 6. Recovery Focus on using AEDs quickly and working as a team.	76
	New Terminology: breaths and lay rescuer	"Rescue breaths" → "breaths." "Bystander" → "lay rescuer."	
	Breaths Terminology	Use "breaths" for lay responders; "ventilations" is for healthcare providers.	
	CPR first, AED early	Start chest compressions immediately, then use the AED as soon as it arrives.	
	Special circumstances	Know that CPR may need quick adjustments for: • opioid overdose • pregnancy • electrocution • hypothermia • drowning Always begin with early CPR and early AED.	NEW
	Hands On CPR	If you cannot or do not want to give breaths, you may perform Hands-Only CPR.	82
	Breaths First	Only give breaths first in likely hypoxia emergencies: • drowning • opioid overdose • pediatric cases Otherwise, always start with compressions.	
	Sequence: Check–Call–Compress–Ventilate–AED	Call 911, start compressions right away, add breaths if trained, use AED ASAP.	81

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Module 14: CPR and AED	Quality & feedback devices	Push hard, fast, and allow full recoil. Minimize pauses. Use real-time feedback devices when available.	81
	When to Stop CPR	Stop only when: • you see clear signs of life • AED says “analyzing” • EMS takes over • you are physically unable to continue	
	Systems of care	Understand the full pathway: lay rescuer → EMS → hospital → recovery. Emphasizes teamwork, fast EMS activation, and AED/naloxone access.	NEW
	Community & public access	Supports: • wider naloxone access • emergency alert technologies to help CPR/AED response • community CPR training and readiness programs	NEW
	Ethical considerations	Rescue actions should respect dignity and Good Samaritan protections. Explains when CPR may be stopped or withheld and the emotional impact on rescuers.	NEW
	Consent, confidentiality, DNR awareness	Students must understand that consent and privacy are essential when giving first aid. They should also be aware that DNR orders apply only in healthcare settings and must follow local policies.	NEW
	On-scene debrief & microlearning	Encourages short debrief after the event and regular microlearning refreshers.	81
Module 15: Choking	Adult/child choking algorithm	For a conscious adult or child: Alternate 5 back blows and 5 abdominal thrusts. If the person becomes unresponsive, begin CPR immediately.	84 - 86
	Infant choking algorithm	Use 5 back blows followed by 5 chest thrusts. If the infant becomes unresponsive, start CPR right away.	